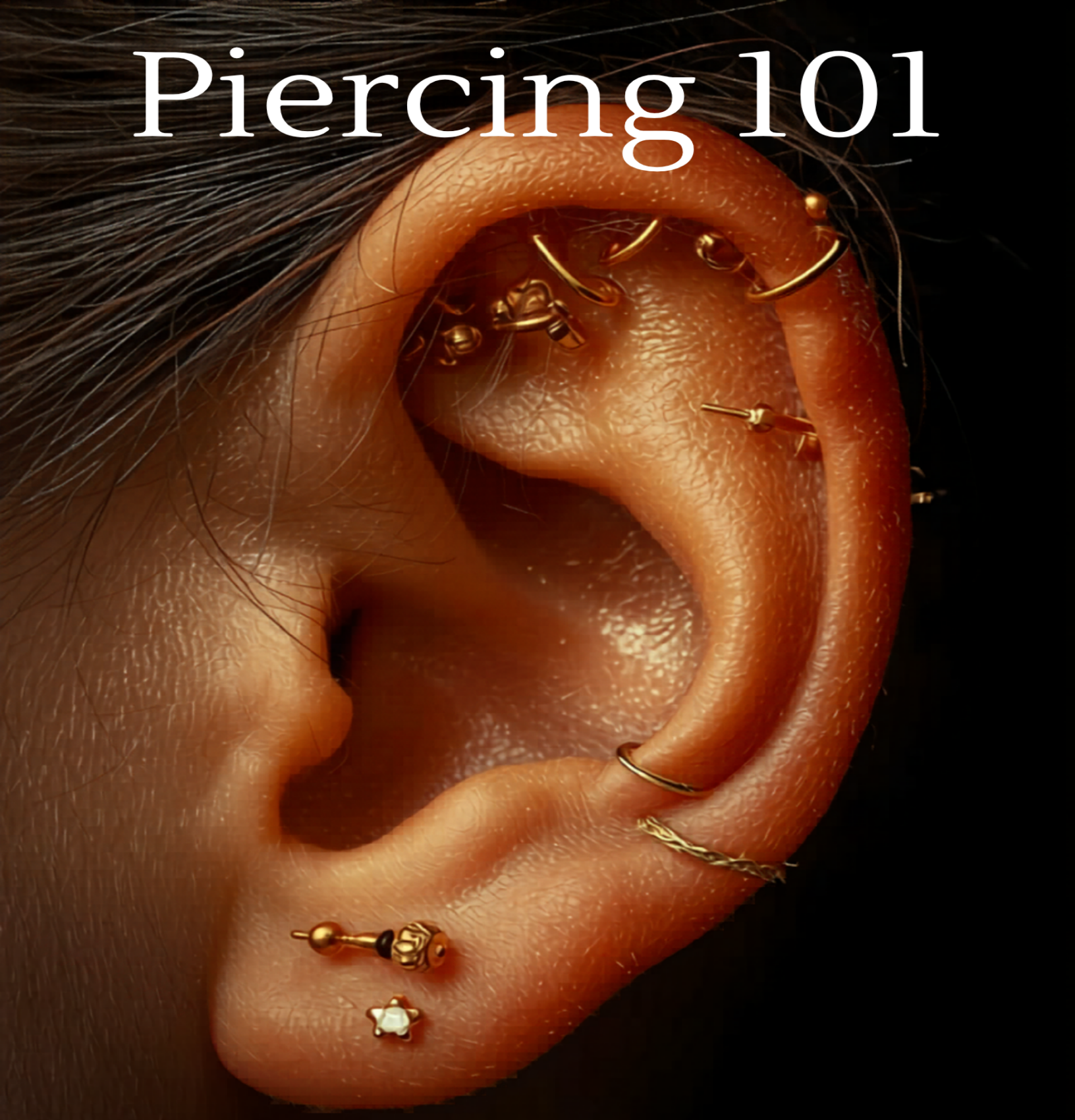
Piercing 101



FREE

What They Don't Tell You

BY RICK ANGELL

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INTRODUCTION

The Truth About This Industry

Why Most Studios Fail the Consumer — And How to Protect Yourself

Here is what the body piercing industry does not want you to know: **there is no mandatory national licensing standard for body piercers in the United States.** In most states, anyone can pick up a needle, rent a chair, and start piercing people tomorrow. No required training. No competency exam. No minimum standard of hygiene or material knowledge. Nothing.

That is the reality you walk into every time you enter a studio you have not vetted. The consequences of choosing the wrong studio are not just cosmetic. They include permanent scarring, life-altering infections, embedded jewelry that must be surgically removed, nerve damage, and documented cases of serious systemic illness.

The Association of Professional Piercers (APP) exists precisely because of this gap. It is a voluntary organization of piercers who hold themselves to standards that far exceed what any government currently requires. The gap between an APP-standard studio and a typical mall kiosk or budget shop is not small — it is enormous.

The Uncomfortable Reality

- The APP estimates that a significant majority of active piercing studios in the U.S. do not meet professional safety standards.
- Piercing guns — still used in mall kiosks, pharmacies, and countless studios — cannot be adequately sterilized and cause blunt force trauma to tissue every single time.
- The most common starter jewelry sold in budget studios contains materials the APP explicitly prohibits: mystery metals, acrylic, plated alloys, and ungraded steel.
- Most piercing "infections" seen by medical professionals were preventable with proper studio choice and aftercare.
- Many piercers have never heard of the APP, never completed a formal apprenticeship, and have never updated their aftercare knowledge beyond what they were told years ago.

How to Use This Guide

- Read Chapters 1–4 before booking any appointment.
- Use Chapter 4's Q&A; table as a literal script. Screenshot it. Bring it with you.
- Use Chapters 6–8 throughout your healing journey as your ongoing reference.

- Use the new chapters on Pain, Scarring, Rejection, and Healing Timelines whenever you have a question during healing.
- Share this guide. The more educated clients are, the higher the standard rises for the entire industry.

CHAPTER 1

Studio Red Flags: The Walk-In Test

You can learn an enormous amount about a studio before you speak to a single person.

The 60-Second Studio Walk-In Test

Observe before speaking to anyone. **More than 2 red flags = leave.**

CHECK	✓ PROFESSIONAL STUDIO	✗ RED FLAG
Smell	Neutral / clean antiseptic odor	Musty, dirty, or strong chemicals masking other odors
Surfaces	Clean, organized, uncluttered	Visibly dirty, stained, or heavily cluttered
Jewelry Display	Sealed sterile packaging or locked cases	Open bowls, loose trays, no packaging
Licenses	Health and business license posted	Nothing displayed or staff avoids question
Autoclave	Visible, labeled sterilizer unit	Not visible or staff doesn't know what it is
Sharps Disposal	Labeled biohazard container visible	No sharps container anywhere
Gloves	In use or clearly available	Bare-handed interactions throughout
Staff Response	Welcomes questions, no pressure	Dismissive, evasive, or high-pressure sales

The Piercing Gun: A Non-Negotiable Dealbreaker

If you see a piercing gun anywhere in a studio — on the counter, in a drawer, anywhere — **walk out.** This is not an exaggeration and not a matter of preference. Piercing guns are prohibited by the APP for every body piercing, including earlobes, and the science is unambiguous.

✗ PIERCING GUN	✓ HOLLOW NEEDLE
Forces blunt stud through tissue — TEARS, doesn't cut. Cannot be sterilized. Causes blunt force trauma. Destroys cell structure. **Banned by APP for ALL piercings.**	Sharp hollow tip removes a clean cylinder of tissue. Single-use, pre-sterilized. Minimal trauma, clean channel. **Required by APP for ALL body piercings.**

The "But It's Just Earlobes" Myth

- Piercing guns are not safe for earlobes. The APP prohibits gun use for ALL piercings including earlobes. Earlobe tissue is still living skin with blood supply and nerve endings. Blunt force trauma is blunt force trauma regardless of location.
- Cartilage (helix, tragus, rook, daith) is even more dangerous. A gun used on cartilage can shatter the structure and cause severe, permanent deformity. This is documented in medical literature.
- Any studio using a gun on cartilage is not just substandard — they are creating foreseeable, preventable harm.

Sterilization: What to Look For and What to Ask

An autoclave is a pressurized steam sterilization unit. It is the only method approved for sterilizing reusable piercing instruments. If you cannot see one — or if staff cannot tell you where it is and when it was last spore-tested — **leave.**

Spore testing verifies that an autoclave is actually achieving sterilization. A biological spore test is run weekly in APP-compliant studios and the results logged. Ask: *"Do you spore test your autoclave and how often?"* A professional will answer immediately. Hesitation or confusion is your answer.

Single-use items — needles, receiving tubes, gloves — must be opened from sealed sterile packaging in front of you. You should watch this happen. If a needle is already out of its packaging before you sit down, that is unacceptable.

Why Cheap Piercings Are the Most Expensive

When a studio's prices are dramatically below the local average, there is always a reason. That reason is almost never "they are passing savings on to you." It is one or more of: substandard jewelry, inadequately trained staff, shortcuts in sterilization, or a business model that profits from upselling.

- Implant-grade titanium, solid gold, and ASTM F138 steel cost significantly more than mystery metal. A studio using real materials cannot sustainably price at $20 per piercing.
- A $25 piercing is rarely $25. It is $25 plus the cost of everything that follows.
- Social media popularity is not a safety credential. A studio can have 200,000 Instagram followers and still harm you. Verify independently using the criteria in this guide.

CHAPTER 2

Piercer Red Flags

The Person Behind the Needle

What Legitimate Training Actually Looks Like

There is no nationally standardized piercing license in the U.S. "Licensed piercer" may mean a local health department permit, a weekend course, a two-year apprenticeship, or nothing at all. Legitimate training looks like this:

- **Formal apprenticeship. The industry standard is a supervised apprenticeship of at least one to two years under an experienced, APP-standard piercer.**
- **APP membership. The APP requires members to demonstrate knowledge of sterilization, anatomy, materials, and safe piercing practices before granting membership.**
- **Bloodborne Pathogens (BBP) certification. Every professional piercer should hold current certification and be able to show you documentation.**
- **Continuing education. Active APP members attend the annual conference with seminars on anatomy, materials science, and technique.**

The Consultation: What Must Happen Before the Needle

A professional piercing appointment does not begin in the piercing room. It begins with a consultation. Your piercer must review your health history, assess your anatomy, discuss placement, show you the specific jewelry, and walk you through aftercare — **all before any needle comes out.**

Piercer Red Flags: What Should Stop You Cold

- **No health consultation whatsoever. Skipping health history is negligent.**
- **Touching you without gloves. Bare hands on a client is unacceptable.**
- **Marking placement without showing you. Placement is your decision.**
- **Not cleaning the skin before piercing. The area must be cleaned with an appropriate antiseptic.**
- **Recommending anything other than sterile saline. Especially alcohol, hydrogen peroxide, Bactine, Neosporin, Dial soap, or tea tree oil.**
- **Telling you to rotate or twist your jewelry. This advice was debunked decades ago. It tears healing tissue and introduces bacteria.**

- **Cannot name the jewelry material and grade. If they don't know what they're putting in your body, don't let them put it there.**
- **Dismissive of your questions. A professional is never annoyed by an informed client.**
- **Rushing the procedure. Proper technique takes time.**

The Single Best Screening Question

Before you book, call or message the studio and ask:

"What aftercare do you recommend?"

The only correct answer per APP standards:
Sterile saline solution (0.9% NaCl, no additives), 1–2 times per day, nothing else, do not rotate the jewelry.

Any other answer tells you everything you need to know — without spending a dollar.

CHAPTER 3

The Jewelry Deep Dive

Materials, Metals & What Goes in Your Body

The jewelry placed in a fresh piercing is not a fashion accessory — **it is a medical implant.** Your body will be in direct, continuous contact with this material for months, sometimes over a year. The wrong material actively causes harm: allergic reactions, chemical leaching, bacterial colonization, and tissue damage that can leave permanent scarring.

Material Safety — Traffic Light Guide (APP Standard)

✓ SAFE — APP APPROVED

- Implant-Grade Titanium (ASTM F136) — best choice for almost everyone
- Implant-Grade Steel (ASTM F138) — confirm grade, not just "surgical steel"
- Solid 14k or 18k Gold (yellow or rose) — must be solid, not plated
- Niobium — nickel-free, excellent for sensitive clients
- Implant-Grade Glass (borosilicate/quartz) — for healed piercings
- Platinum — excellent biocompatibility

■ ASK DETAILED QUESTIONS

- "Surgical Steel" without ASTM F138 certification — unregulated term
- White Gold — may be nickel-alloyed; demand alloy breakdown
- Organic materials (wood, bone, horn) — ONLY in fully healed piercings

✗ NEVER — WALK AWAY

- Mystery metal / "hypoallergenic" with no specification
- Acrylic or plastic — porous, leaches chemicals, cannot be sterilized
- Sterling silver — copper content oxidizes and permanently stains tissue
- Gold-plated / gold-filled / gold-vermeil / titanium-coated
- Brass, nickel, or any unknown base metal

Material by Material: What Each One Actually Means

Implant-Grade Titanium — ASTM F136

The most recommended material for initial piercings. Completely nickel-free. Biocompatible with living tissue — the same class of material used in orthopedic implants and surgical hardware. The designation ASTM F136 is the

specific certification grade. When a studio says "titanium" without specifying ASTM F136, ask for clarification. Titanium can be anodized to create color — pink, blue, purple, teal — all still biocompatible and safe.

Implant-Grade Steel — ASTM F138

Acceptable for most clients, but important caveat: implant-grade steel contains a small amount of nickel (approximately 10–14%) locked into the alloy. For clients without nickel sensitivity, this is generally fine. "Surgical steel" is a marketing term with no regulated standard — always ask for ASTM F138 specifically.

Solid 14k or 18k Gold

Excellent material with significant fine print. It must be solid. Gold-plated, gold-filled, and gold-vermeil are never acceptable for initial piercings. 14k and 18k are appropriate. White gold is frequently nickel-alloyed. Rose gold is typically copper-alloyed (no nickel) and is generally safe.

Niobium

An elemental metal (not an alloy) that is completely nickel-free, naturally biocompatible, and can be anodized for color. An excellent option for clients with metal sensitivities. Less commonly stocked than titanium but available at any quality studio.

Threading: The Detail That Separates Professional from Amateur

✗ EXTERNALLY THREADED	✓ INTERNALLY THREADED
Threads on the outside of the post drag through the healing channel. Tears tissue. Deposits bacteria with every movement. **Not acceptable for initial piercings.**	Threads are INSIDE the post. End piece screws inward. Only smooth metal contacts healing tissue. **APP standard for all initial piercings.**

CHAPTER 4

The Questions You Must Ask

Screenshot this chapter. Bring it to your appointment.

STERILIZATION & STUDIO

Q: What sterilization method do you use?

✓ Autoclave. Instruments individually packaged, full cycle before each use. Weekly spore tests on file.

✗ Disinfectant spray / UV light / Barbicide / bleach solution.

Q: Can I see the needle before you open it?

✓ Yes — shown sealed, single-use, from reputable manufacturer (e.g. Kai).

✗ It's already set up / we pre-open for efficiency.

Q: Do you do autoclave spore testing and how often?

✓ Yes, weekly. We keep records on file and can show you.

✗ We clean it regularly / What's a spore test?

Q: What do you use to clean the skin before piercing?

✓ An appropriate antiseptic — typically chlorhexidine gluconate or similar.

✗ Alcohol wipe only / nothing / soap and water.

JEWELRY & MATERIALS

Q: What material exactly is this jewelry?

✓ ASTM F136 implant-grade titanium / ASTM F138 / solid 14k gold / niobium. Can show supplier documentation.

✗ It's hypoallergenic / it's surgical steel / I'm not sure of the grade.

Q: Is this internally threaded or threadless?

✓ Yes — all starter jewelry is internally threaded or press-fit threadless.

✗ I don't know what that means / does it matter?

Q: Do you use a piercing gun for any piercings?

✓ Never. Exclusively single-use, sterile hollow needles for every piercing.

✗ Only for earlobes / guns are fine if cleaned.

Q: Is your gold solid or plated?

✓ Solid 14k or 18k only. We don't carry plated, filled, or vermeil for initial piercings.

✗ Gold-filled is basically the same / gold-plated is fine.

TRAINING & CREDENTIALS

Q: Are you APP-member or do you follow APP standards?

✓ Yes, I'm an APP member / I follow APP standards for materials, sterilization, and aftercare.

✗ I've been piercing for years / I don't need a membership / what's the APP?

Q: Where did you train and how long was your apprenticeship?

✓ Formal apprenticeship under [name] for [X time]. Also completed [courses/conference training].

✗ I'm self-taught / I took a weekend course / I learned informally.

Q: Do you have a current bloodborne pathogens certification?

✓ Yes, I renew it annually. Happy to show documentation.

✗ I used to have one / do I need that for piercing?

AFTERCARE

Q: What aftercare do you recommend?

✓ Sterile saline solution only — 0.9% NaCl, no additives, 1–2 times per day. Do not rotate the jewelry.

✗ Alcohol / hydrogen peroxide / Bactine / Neosporin / tea tree oil / twist it twice a day.

Q: Should I rotate my jewelry?

✓ Absolutely not — ever. Leave the jewelry completely still. That advice is outdated and causes real harm.

✗ Yes, rotate or twist it twice a day / move it when cleaning.

Q: What should I do if I get a bump?

✓ Come back and see me. Most bumps are irritation, not infection. I'll assess the cause and address it.

✗ Put tea tree oil on it / change the jewelry yourself / take it out if it looks infected.

Q: Do you do downsize appointments?

✓ Yes, included or available at minimal cost at 4–8 weeks for most piercings.

✗ You can do it yourself / you don't really need to downsize.

CHAPTER 5

Anatomy & Placement

Why Not Every Piercing Is Right for Every Body

One of the most valuable services a professional piercer provides is an honest anatomy assessment. A piercer who tells you a piercing will not work for your anatomy is protecting you from a piercing that will fail, reject, migrate, or scar. A piercer who does it anyway is taking your money and leaving you with the consequences.

Navel / Belly Button

The navel must have a defined ridge of tissue with enough depth for the jewelry to rest against. If the navel is flat, inverted, significantly folded, or if the tissue pinches when sitting, a traditional navel piercing will almost certainly reject or migrate. A professional assesses this seated and standing, both.

Ear Cartilage — Helix, Tragus, Daith, Rook, Industrial

Every cartilage piercing requires specific anatomical features. A daith requires a pronounced innermost fold of the ear. A rook requires a defined antihelix ridge. An industrial requires two compatible placement points at the correct angle — if the anatomy does not support it, the piercing will migrate, reject, or cause constant irritation.

Nostril

Nostril placement involves assessing the thickness and position of the ala (nostril rim), the interior of the nose at the placement point, and how the jewelry will sit both externally and internally. Placement too high, too low, or too far inward creates problems that are difficult to correct once the piercing is established.

Septum

The septum piercing passes through the "sweet spot" — a thin membrane of tissue between the cartilage and the tip of the nose. The size, position, and accessibility of this varies significantly between individuals. Cartilage septum piercings — going through actual cartilage — are extremely painful and almost always indicate improper placement.

Surface Piercings and Dermal Anchors

Surface piercings carry a categorically higher rejection rate than piercings through natural anatomical features. A reputable piercer will always discuss this honestly. Dermal anchors are not permanent — they migrate or reject over time.

Tongue

The underside of the tongue contains two prominent veins that must be identified and avoided. Placement requires visual assessment of vein position and evaluation of tongue tissue depth.

When a Piercer Says It Won't Work

Thank them. A piercer who declines a procedure because the anatomy will not support it is doing their job correctly.

Do not go to another studio hoping someone will say yes. If the anatomy is a problem, the anatomy is a problem regardless of who is holding the needle. The only thing a different answer gets you is the same failure, just paid for twice.

CHAPTER 6

Healing: The Full Truth

A piercing that looks healed on the outside is not necessarily healed.

Most studios dramatically underestimate healing times. Clients are told their cartilage is healed at three months when it is not. Healing occurs from the outside in. The surface closes first. The tissue channel inside takes far longer. Acting on surface appearance is one of the most common causes of piercing complications in existence.

Full Healing Time Chart (APP Guidelines)

PIERCING TYPE	HEALING RANGE	NOTES
Earlobe	1–6 months	Fastest-healing piercing for most people
Tongue	2–4 months	Fast surface heal; strict oral hygiene required
Septum	1–6 months	Sweet-spot only; cartilage placement takes much longer
Lip / Labret	3–6 months	Oral hygiene critical; avoid irritating foods
Nostril	4–12 months	Surface closes quickly; internal takes much longer
Dermal Anchor	2–6 months	Higher rejection rate; monitor placement
Daith	6–18 months	Dense cartilage; one of the slower-healing ear piercings
Helix / Flat	6–18 months	Do not sleep on it; use travel pillow
Tragus	6–18 months	Avoid earbuds during healing
Rook	6–18 months	Dense cartilage; patience required
Nipple	6–18 months	Loose clothing; avoid pressure from bras
Industrial	6–24 months	Two piercings; double the healing complexity
Navel	6–24 months	High failure rate if anatomy not properly assessed
Surface Piercing	6–24 months	Highest rejection rate of any piercing category

The Three Stages of Healing

Stage 1: Inflammatory Phase — Days 1–10

Your body sends blood flow, white blood cells, and inflammatory fluid to the wound site. Swelling, redness, and warmth are expected. Lymph fluid — the clear or slightly white fluid that dries into crust around your jewelry — is completely normal. It is not infection. It is not a problem. Do not pick at it.

Stage 2: Proliferative Phase — Weeks 2–12+

New tissue cells begin forming along the piercing channel. The surface may look and feel completely fine. The interior is still raw, fragile, and actively forming. This is the phase during which most people incorrectly believe they are healed — and cause damage by changing jewelry, going swimming, or stopping aftercare.

Stage 3: Maturation and Remodeling — Months to Years

The epithelial lining strengthens and the fistula (piercing channel) develops its full structure. This takes far longer than most people are told. A navel piercing that "feels fine" at six months may still have twelve or more months of maturation ahead of it.

What Slows or Stops Healing

- Changing jewelry before healing is complete — the single most common cause of piercing failure
- Sleeping directly on the piercing — use a travel pillow with the ear in the open center
- Poor nutrition — protein, vitamin C, and zinc are critical for tissue formation
- Stress and sleep deprivation — cortisol suppresses immune function, and healing is an immune activity
- Smoking and alcohol — impair blood flow and tissue oxygenation
- Touching with unwashed hands — every unnecessary contact introduces bacteria
- Over-cleaning — more than twice a day strips beneficial flora and moisture
- Non-approved products — alcohol, peroxide, and essential oils kill healing cells
- Ill-fitting clothing or accessories that catch, pull, or press on the piercing

CHAPTER 7

Aftercare: The Right Way

Sterile saline. Twice a day. Leave it alone.

The APP has refined its aftercare guidelines over decades. The current standard is simple: sterile saline, twice a day, leave it alone. That is it. Everything else is noise — and some of it is actively harmful.

The Official APP Aftercare Standard

- Sterile wound wash saline — 0.9% NaCl, absolutely no additives
- 1–2 times per day only. Over-cleaning is as damaging as under-cleaning.
- Pat dry with clean gauze or a fresh paper towel. Never cloth towels.
- Leave the jewelry completely still. Never rotate, twist, or move it.

That is the complete protocol. Nothing else belongs in or on a healing piercing.

Step-by-Step Daily Routine

Step 1: Wash Your Hands

Before any contact with your piercing, without exception, wash your hands with soap and water for a minimum of 20 seconds.

Step 2: Apply Sterile Saline

NeilMed Wound Wash (0.9% NaCl, no additives) is the most widely recommended product. Spray directly onto the piercing or saturate clean gauze and hold against the site for 1–2 minutes. This softens and loosens dried lymph fluid without harming healing tissue.

Step 3: Gentle Rinse

Allow the saline to rinse away loosened crust. If crust does not come away easily, do not force it. Firmly attached dried lymph will soften in the shower and rinse away naturally.

Step 4: Pat Dry

Clean gauze or a fresh paper towel only. Cloth towels harbor bacteria and snag jewelry. Pat gently — never rub or wipe.

Step 5: Leave It Alone

Walk away. Do not check it, touch it, twist it, or apply anything else.

Step 6: Repeat Once More Today

Morning and evening. That is your complete routine. For oral piercings, add an alcohol-free mouthwash rinse after meals and before bed.

Products to Never Use — And Why

PRODUCT	WHY IT HARMS YOUR PIERCING
Rubbing Alcohol (Isopropyl)	Kills fibroblasts — the cells responsible for building new tissue — as effectively as it kills bacteria. Cytoto
Hydrogen Peroxide	Produces a free radical reaction that destroys healthy tissue alongside bacteria. The bubbling you see is
Bactine	Contains benzalkonium chloride (cytotoxic antiseptic) and lidocaine, which masks pain signals telling you
Neosporin / Antibiotic Ointments	Creates an oxygen-free environment that promotes anaerobic bacterial growth. Also associated with cor
Tea Tree Oil	Cytotoxic to human skin cells at concentrations as low as 2–5%. Multiple peer-reviewed studies confirm
Essential / Coconut / Vitamin E Oil	None are sterile. They introduce organic material into a healing wound and can trap bacteria.
Soap directly in the channel	Use soap only on surrounding skin during showering, not inside the wound channel. Soap residue cause
Alcohol-based mouthwash	Too harsh for daily use on healing oral tissue. Use alcohol-free formulations only.

CHAPTER 8

Infection vs. Irritation

The distinction that changes everything.

The vast majority of piercing "problems" that people experience are not infections — they are irritation reactions. Treating irritation as infection with antibiotics, alcohol, or jewelry removal will worsen the situation in almost every case.

	✗ INFECTION	✓ IRRITATION BUMP
Redness	Spreading outward from site	Localized, right at the bump
Discharge	Yellow or GREEN pus	Clear or white lymph fluid — normal
Odor	Foul smell	None whatsoever
Pain	Worsening, throbbing	Mild, manageable tenderness
Heat	Extends well beyond site	Slightly warm at bump only
Systemic	Fever, chills, fatigue possible	No systemic symptoms at all
Timeline	Gets worse day over day	Stable or slowly improving
Action	See a doctor — KEEP JEWELRY IN	Remove irritant, continue saline

Common Causes of Irritation Bumps

- Sleeping directly on an ear piercing without a travel pillow
- Jewelry that is too short — the most common cause of bumps in fresh piercings
- Snagging jewelry on hair, clothing, pillowcases, headphones, or masks
- Touching with unwashed hands
- Any aftercare product other than sterile saline — especially tea tree oil or alcohol
- Swimming in pools, hot tubs, lakes, or oceans
- Changing jewelry before healing is complete

How Long Should You Try Before Seeking Help?

Week 1: Identify and remove every possible irritant. Switch to sterile saline only. Stop all other products and touching.

Week 2: The bump should be stable — not necessarily gone, but not larger. If it is visibly growing, see your piercer this week.

Weeks 2–4: With the irritant removed, most irritation bumps begin reducing. If the bump is the same size at four weeks with the irritant fully removed, see your piercer for a jewelry assessment.

Beyond 4 weeks: A bump that has not reduced at all after four weeks of correct care warrants professional assessment.

Critical Instructions If You Suspect Infection

DO NOT REMOVE THE JEWELRY.

Removing the jewelry traps the infection inside a closing pocket of tissue. This can develop into an abscess requiring surgical drainage. The jewelry keeps the channel open and allows drainage to occur.

- See a medical professional — urgent care or your physician
- Tell them explicitly that the jewelry must remain in place
- Seek emergency care immediately if you develop a high fever, spreading redness (cellulitis), or red streaks extending from the piercing site

CHAPTER 9

Your Rights as a Client

Entering a piercing studio is consenting to an invasive procedure. You have rights.

Full Consultation Before Any Procedure

No needle comes out until your health history has been reviewed, anatomy assessed, placement discussed, and you have been fully informed of the procedure, jewelry, and aftercare. This is not a courtesy — it is the minimum standard of professional practice.

The Right to See and Inspect Your Jewelry

You are entitled to examine the specific jewelry that will be used in your piercing, in its sterile packaging if applicable, before the procedure begins. You may reject jewelry you are not comfortable with.

Control Over Placement

Placement is your body and your decision. Your piercer marks, shows you in a mirror, explains the reasoning, and waits for your explicit verbal approval before proceeding.

The Right to Withdraw Consent at Any Time

Before the needle, during marking, mid-procedure — whenever. No explanation required. A professional respects this immediately and without comment.

To Watch the Needle Be Opened

Single-use needles must be opened from sealed sterile packaging in your presence. You have the right to watch this happen.

Written Aftercare Instructions

You leave every appointment with written aftercare instructions. Not verbal. Written. If a studio cannot provide this, they are not operating professionally.

Transparent Pricing Before You Sit

Complete cost — service fee, jewelry cost, any expected follow-up costs — communicated before you sit in the chair. No surprise charges.

Clean, Gloved, Professional Treatment

Your piercer wears gloves from the moment they begin your procedure. The room is clean. The equipment is sterile. Non-negotiable.

Honest Anatomy Assessment

If a piercing is not viable for your anatomy, you have the right to be told that honestly. Proceeding without disclosure is an ethical violation of client trust.

How to Report a Studio or Piercer

- **Local health department:** File a complaint for health, safety, or sanitation violations.
- **The APP (safepiercing.org):** Report studios or piercers misrepresenting APP membership.
- **Google and Yelp:** An honest, factual public review protects other consumers and creates accountability.
- **Your state body art licensing board:** Many states regulate body art practitioners. Search your state + "body art board."

CHAPTER 10

Finding a Good Piercer

The APP, The Piercing Bible & Resources

The Association of Professional Piercers (APP)

The APP is the only international nonprofit organization dedicated exclusively to the health, safety, and education standards of professional body piercers. Membership requires demonstrating knowledge of sterilization, material standards, and safe practices. Members commit to continuing education and published standards.

Visit safepiercing.org to:

- Find APP member piercers in your area using the member locator tool
- Download the official APP aftercare guidelines for free
- Access the approved initial jewelry material list
- Read educational resources on piercing safety and standards
- Report a studio misrepresenting APP membership

The Piercing Bible by Elayne Angel

The Piercing Bible: The Definitive Guide to Safe Body Piercing by Elayne Angel is the comprehensive reference that every serious piercing professional and informed client should own. Elayne Angel is one of the most experienced and respected piercers in the world. The book covers anatomy, procedure, healing, jewelry, troubleshooting, and virtually every piercing type. Available through Amazon and major booksellers.

Additional Resources

NeilMed Wound Wash — Most pharmacies and online retailers. 0.9% NaCl, no additives. Look for the wound wash specifically, not the sinus rinse.

Your Piercer — A reputable piercer wants to hear from you throughout healing. Bring concerns early. They can assess in person what no guide can replicate remotely.

Your Primary Care Physician — For genuine medical concerns. If you suspect infection with systemic symptoms, see a doctor.

"An informed client is the best client — and the best thing that can happen to the professional piercing industry."

★ NEW CHAPTER

Pain Scale by Piercing Type

An honest guide to what to expect — and what affects your experience.

One of the most common questions piercers receive is also one of the least honestly answered: *Does it hurt?* The honest answer: yes, briefly, and the level varies significantly by location, anatomy, technique, and your individual nervous system response. A well-executed professional piercing is almost always described as a brief, sharp sensation followed by pressure, followed by it being over.

PIERCING	RANGE (1–10)	WHAT CLIENTS REPORT
Earlobe	**1–3**	Brief pinch. Most report surprise at how mild it is.
Eyebrow	**2–4**	Less nerve density than expected. Often surprises with mildness.
Nostril	**3–5**	Sharp sting, eyes may water reflexively — nasal reflex, not pain.
Septum	**3–6**	Sweet-spot placement: low pain. Cartilage hit: significant pain.
Tongue	**3–5**	Clamp pressure often noted more than needle. Swelling after is notable.
Navel	**3–5**	Usually described as a pinch. Anatomy matters more than pain here.
Helix / Flat	**3–5**	Pressure more than pain. Dull ache for hours after.
Lip / Labret	**3–5**	Brief and sharp. Lip swelling after is notable.
Conch	**4–6**	Dense cartilage. Similar experience to helix.
Daith	**4–6**	Inner ear amplifies sensation. Pressure-heavy.
Tragus	**4–6**	Crunching sensation is common — not harmful, just cartilage density.
Surface Piercing	**4–6**	Pinching sensation as skin is tented. Varies heavily by site.
Dermal Anchor	**4–7**	Depends on method. Needle method preferred by APP piercers.
Rook	**5–7**	Dense cartilage. One of the higher reported pain points in ear work.
Industrial	**5–7**	Two separate punctures. Each is brief, but second follows first.
Nipple	**5–8**	High nerve density. One of the most variable reported pain points.

What Affects Pain Level

- **Technique.** A sharp, correctly sized hollow needle used by a skilled piercer produces dramatically less trauma than poor technique. This is the largest variable within your control — choose your piercer carefully.

- **Breathing.** A breath-in on the needle and breath-out on the pierce genuinely reduces tension in surrounding tissue and perceived pain.
- **Anxiety.** Anticipatory anxiety elevates cortisol, which heightens pain sensitivity. Tell your piercer if you are anxious — a professional will slow down.
- **Blood sugar.** Do not fast before your appointment. Low blood sugar significantly increases the likelihood of a vasovagal response (lightheadedness, fainting).
- **Menstrual cycle.** Pain sensitivity varies across the cycle. Some clients report higher sensitivity in the days before menstruation.
- **Alcohol.** Never drink before a piercing. Alcohol thins blood, impairs healing, and does not reduce pain. Most professional studios will decline to pierce someone who has been drinking.

★ NEW CHAPTER

Keloid vs. Hypertrophic Scarring

Two different problems. Two completely different responses.

These two types of scarring are confused constantly — by clients, and even by some medical professionals unfamiliar with piercing. They look similar. They are not. The cause, behavior, and correct response are entirely different. Treating one as the other almost always makes the situation worse.

	HYPERTROPHIC SCAR	KELOID
Appearance	Raised, firm bump. Stays within the wound boundary.	Raised growth that extends beyond the original wound site.
Color	Pink to red. Fades over time.	Pink, red, or darker than surrounding skin. Does not fade.
Growth	Stable or shrinks over time	Actively grows — may continue expanding for months or years
Boundary	At the piercing site only	Spreads beyond the piercing, sometimes significantly
Cause	Irritation, trauma, pressure, improper jewelry	Genetic predisposition to collagen overproduction
Who Gets Them	Anyone — irritation is universal	People with a genetic tendency. More common in darker skin tones
Treatment	Resolves when irritant is removed	Requires medical intervention
First Action	Identify and remove the irritant. Continue saline.	Consult a dermatologist or plastic surgeon. Do not attempt home tr

The Hypertrophic Scar: What You're Probably Looking At

The vast majority of bumps and raised tissue around piercings are hypertrophic — not keloid. This means they are irritation responses, not permanent overgrowth, and they respond well to proper care.

The correct response is not to treat the bump — it is to find and eliminate the cause. Return to your piercer for a jewelry assessment, switch to sterile saline only, use a travel pillow if sleeping on an ear piercing, and stop touching it. Most hypertrophic scars resolve in two to eight weeks once the irritant is consistently removed.

Warning: Treating them with tea tree oil, alcohol, or harsh products will worsen them every time.

The Keloid: A Different Situation Entirely

A keloid is not an irritation response — it is a genetic one. The body does not stop producing collagen when the wound closes. It continues. The scar grows beyond the wound boundary and may grow for months or years.

If you have a known keloid history, this must be disclosed to your piercer before any procedure. A responsible piercer will discuss the risk honestly — certain locations carry higher keloid risk, and in some cases a piercer may decline a site where keloid formation is highly likely.

Do not attempt to remove a keloid at home. Do not apply harsh products.
Cutting into a keloid without proper treatment often causes it to regrow larger.

Keloids require evaluation by a dermatologist. Medical options include corticosteroid injections, silicone sheeting, laser therapy, and surgical excision combined with treatments to prevent regrowth.

★ NEW CHAPTER

Migration & Rejection

Recognizing what's happening while you can still act.

Rejection is one of the most misunderstood piercing outcomes — and one of the most preventable if caught early. A piercing that is actively rejecting gives you warning signs. Knowing what they look like means you can intervene before significant scarring occurs.

What Is Rejection?

Your immune system classifies your jewelry as a foreign object. In most cases, with proper jewelry material, sizing, and placement, the body builds a stable tissue channel around the jewelry. In rejection, the body instead attempts to push the foreign object out through the skin — the same way it would push out a splinter — by thinning the tissue between the jewelry and the skin surface.

Caught early, a rejecting piercing can be removed before significant scarring occurs. Left too long, the migration path becomes a permanent visible scar channel.

Warning Signs — In Order of Progression

STAGE 1 — Early Signs (Act Now)

• The skin between entry and exit holes appears thinner than when you were first pierced • You can see the bar of the jewelry through the skin — a visible outline that was not there before • The jewelry seems to be sitting higher or closer to the surface than originally placed • The distance between the two ends appears to have decreased

STAGE 2 — Active Rejection (See Your Piercer Today)

• The jewelry is clearly visible through translucent-looking skin • The holes have moved noticeably from their original position • Redness, flaking, or dry skin over the bar between the holes • The jewelry feels like it is just under the surface rather than through deeper tissue

STAGE 3 — Advanced (Remove Immediately)

• One or both ends of the jewelry are being pushed through the skin • The skin over the bar has broken down or is actively peeling • The jewelry can be seen almost completely

What to Do

If you recognize early signs, see your piercer — do not wait. If rejection is confirmed, the correct action is almost always to remove the jewelry before it exits on its own. A controlled removal at Stage 1 or 2 leaves a small, often barely noticeable scar. A full rejection that runs its course leaves a visible scar channel.

- **Do not attempt to push the jewelry back into position**
- **Do not change to heavier jewelry hoping it will "hold"**
- **Do not apply products to the skin over the bar**

Piercings Most Prone to Rejection

- Surface piercings — highest inherent rejection risk of any category. A professional will tell you this before you commit.
- Jewelry that is too thin (fine-gauge has less tissue to anchor through)
- Incorrect jewelry type for the placement (curved barbell in a surface piercing — see Chapter 3)
- Non-implant-grade materials that cause ongoing immune reaction
- Placement in areas of constant movement, friction, or pressure (waistband, wrist, collarbone)
- Navel piercings in anatomy that does not support them

★ NEW CHAPTER

When to Retire a Piercing

Letting go at the right moment means a smaller scar and a cleaner outcome.

This is the chapter most piercing guides skip. One of the most common ways a minor problem becomes a significant scar is a client holding on to a piercing that the body has made clear it does not intend to keep. Retiring a piercing is not failure — it is the right clinical decision.

Signs That a Piercing Should Be Retired

Active, progressive rejection

If your piercing is showing Stage 2 or 3 rejection signs, removal before full rejection is almost always the right decision. The scar from a controlled retirement is significantly smaller than the scar from a complete rejection.

Jewelry that has embedded or is embedding

If the end of your jewelry is sinking into the tissue — you can no longer see one or both ends — this is a medical situation. See a piercer or doctor immediately. Do not attempt to pull it out yourself.

Chronic irritation that has not resolved despite correct care

If you have eliminated every known irritant, been using only sterile saline, seen your piercer for jewelry adjustments, and waited an appropriate amount of time — and the piercing is still causing ongoing problems — the anatomy may genuinely not support that placement.

Significant lifestyle conflict

A piercing that cannot be protected during your work, sport, or daily activity will be in a chronic state of trauma and will not heal. Early retirement before significant damage is wiser than years of struggling healing.

Post-pregnancy or significant body composition changes

Navel piercings in particular often do not survive pregnancy intact. If the jewelry has migrated significantly or the anatomy of the site has changed, a professional assessment may recommend retirement and re-piercing once the body has stabilized.

How to Retire a Piercing Correctly

- See your piercer to have the jewelry removed professionally, especially for piercings that have begun embedding or are in a difficult location.

- After removal, clean the site with sterile saline until the holes close.
- Do not pick at or squeeze the closing site.
- Allow full closure before considering re-piercing — ask your piercer how long that should be for your specific site.
- A closed site does not mean fully healed internal tissue. The fistula may persist internally for some time after the surface closes.

★ NEW CHAPTER

Re-Piercing Guide

What you need to know before going back.

Re-piercing is one of the most common questions piercers receive and one of the least covered topics in consumer guides. Whether your piercing closed naturally, you retired it, or it rejected — here is what actually applies.

Can the Same Site Be Re-Pierced?

Often, yes — with important conditions. Scar tissue itself is not necessarily a barrier to re-piercing. Dense scar tissue can be pierced through, but it changes the experience (more resistance, sometimes more discomfort) and affects healing (scar tissue has reduced blood supply, which can slow the process).

Rejection scarring that follows a channel through the tissue is the most complex situation. Your piercer will assess and often recommend adjacent placement slightly away from the scar channel.

How Long to Wait

SCENARIO	MINIMUM WAIT	NOTES
Piercing closed naturally (short-term wear)	3–6 months	Depends on how developed the fistula was
Piercing retired intentionally during healing	3–6 months	Tissue needs time to rebuild
Piercing retired after partial rejection	6–12 months	Scar tissue must fully mature
Full rejection with scar channel	12+ months	Consult piercer; may need longer
Embedded jewelry removal	Consult piercer/doctor	Depends on trauma and wound healing
Piercing closed after years of wear	6+ months	Long-worn fistulas close slowly and unevenly

What Changes the Second Time

- **Scar tissue changes jewelry sizing.** Your piercer will assess — you may need a different gauge or length.
- **Placement may shift.** If the original placement contributed to failure, a professional will recommend adjusted placement.
- **Healing may be slower.** Scar tissue heals differently than intact skin. Budget more time, not less.

- **The anatomy assessment matters even more.** If the original piercing failed due to anatomy, re-piercing the exact same way in the exact same place will produce the same outcome.

Earlobe Re-Piercing: The Special Case

Stretched, torn, or gauge-closed earlobes require individual assessment. A lobe that was stretched to a large gauge has tissue that has remodeled significantly and will not simply return to its pre-stretched state. Whether and where it can be re-pierced depends entirely on what the tissue looks like after closure and healing. Some stretched lobes that have been surgically repaired can be re-pierced; many cannot.

★ NEW CHAPTER

Life During Healing

Your complete lifestyle reference — all in one place.

Water and Swimming

ACTIVITY / SITUATION	GUIDANCE
Shower	Fine. Let warm water run over the piercing naturally. Pat dry after.
Bath	Avoid submerging the piercing.
Swimming pool	Avoid entirely. Chlorine is a chemical irritant; pools harbor bacteria.
Hot tub / spa	Avoid entirely. High bacterial load, chemical exposure, and heat promote complications.
Ocean / lake / river	Avoid entirely. Open bodies of water carry significant bacterial contamination.

Sleep

ACTIVITY / SITUATION	GUIDANCE
On a healing ear piercing	Use a travel pillow (donut-shaped) with the ear in the open center — eliminates pressure.
Pillowcase hygiene	Change every 2–3 days during early healing.
Clothing overnight	Wear loose clothing that does not press on body or surface piercings overnight.

Exercise

ACTIVITY / SITUATION	GUIDANCE
Light walking, yoga, low-impact	Generally fine. Avoid contact with the piercing site.
Contact sport	Consult your piercer. Some piercings should be covered or retired for the sport.
After heavy sweating	Rinse with sterile saline after exercise. Sweat is not inherently harmful but keep the site clean.
Helmets / headgear	Ensure headgear does not press directly on a healing ear piercing.

Diet and Nutrition

Your body builds healing tissue from the food you eat. This is not a minor variable.

- **Protein:** Essential for tissue repair and regeneration. Inadequate protein measurably slows healing.
- **Vitamin C:** Critical for collagen synthesis. Found in citrus, peppers, broccoli, strawberries.
- **Zinc:** Supports immune function and wound healing. Found in meat, legumes, nuts, seeds.
- **Hydration:** Adequate water intake supports blood flow and cellular function throughout healing.
- **Alcohol:** Impairs blood flow and immune function. Moderate intake during active healing.
- **Smoking:** Reduces tissue oxygenation and blood flow. Consistently associated with slower healing.

Stress and Sleep

Healing is an active immune function. Cortisol — the primary stress hormone — suppresses immune activity. Chronic stress and sleep deprivation demonstrably slow wound healing. If you are going through a high-stress period, build in extra time for healing and extra attentiveness to aftercare.

Hair, Makeup, and Skincare

- Long hair can snag in jewelry and cause sudden trauma. Tie hair back during early healing for ear and facial piercings.
- Hair products (dry shampoo, hairspray) should be kept away from healing piercings. Cover nearby piercings when using sprays.
- Keep makeup, foundation, and setting sprays away from healing facial piercings.
- Skincare products with acids, retinoids, or actives should not contact healing piercing sites.

Phones and Headphones

- A phone held against a healing ear piercing creates sustained pressure plus contact from a bacteria-laden surface. Use speakerphone or switch sides.
- Over-ear headphones pressing on healing cartilage piercings are a frequently overlooked cause of irritation bumps. Use earbuds on the healing side, or go without.

★ NEW CHAPTER

Week-by-Week Healing Timeline

What normal looks like at every stage — so you don't panic at expected milestones.

Use this as your reference throughout healing. When in doubt, contact your piercer.

Helix (Ear Cartilage) — Typical Healing: 6–18 Months

TIMEFRAME	WHAT'S NORMAL	WARNING SIGNS
Days 1–3	Redness, warmth, swelling. Clear to white-yellow lymph fluid drying into crust. Tenderness.	Spreading redness beyond the site. Worsening pain after 48 hours. Green discharge.
Week 1–2	Swelling begins to reduce. Crusting continues. Some tenderness when touched.	Fever. Foul odor. Pain intensifying rather than settling.
Weeks 2–6	Surface looks more settled. Crusting reduces. Still tender to pressure. Do not change jewelry.	Bump forming. Jewelry visibly tighter as scar tissue forms around improper fit.
6–8 Weeks	DOWNSIZE APPOINTMENT. Piercer replaces starter jewelry with shorter post. This is a clinical step, not optional.	Do not skip the downsize. Leaving oversized starter jewelry causes continued irritation.
Months 2–6	Continued gradual settling. Minimal discharge. Some days may feel more tender than others — normal fluctuation.	Jewelry visibly moving position (migration). Bump that is growing.
Months 6–12	For most clients, the piercing looks and feels largely normal. Internal tissue is still maturing.	Avoid changing jewelry without piercer confirmation of full healing.
12–18 Months	Full internal healing for most anatomy. Piercing channel is mature and stable.	If still having ongoing issues at 12 months, see your piercer for reassessment.

Nostril — Typical Healing: 4–12 Months

TIMEFRAME	WHAT'S NORMAL	WARNING SIGNS
Days 1–3	Redness, minor swelling. Clear discharge and crusting. Eyes may water on that side — normal nerve response.	Spreading redness. Significant swelling that does not begin reducing after 3 days.

TIMEFRAME	WHAT'S NORMAL	WARNING SIGNS
Week 1–2	Swelling reduces. Inside of nose may feel tender. Some visible lymph discharge.	Any green or yellow pus with foul odor.
Weeks 2–6	Surface begins to look settled. Crusting still present, especially inside nostril.	Bump forming at exit or entry hole. Jewelry seeming shorter as swelling changes fit.
6–8 Weeks	DOWNSIZE APPOINTMENT. Interior post length reduced now that initial swelling has resolved.	Pain during downsize if jewelry has begun embedding — seek care sooner.
Months 2–4	Most clients find the nostril looking healed externally. Interior is still forming. Do not change to a ring yet.	Assuming external appearance means internal completion. It does not.
4–12 Months	Many piercings are functionally healed. For thicker tissue, 12 months is realistic. Your piercer assesses.	Changing jewelry before professional confirmation.

General Rules Across All Healing Timelines

- **Good days and bad days are normal.** A piercing that felt fine last week and is tender this week has not become infected overnight. Healing is not linear.
- **Healing can restart.** One trauma event — a snag, sleeping on it wrong — can restart the inflammatory phase. This is not failure. Eliminate the cause and continue aftercare.
- **"Feels healed" is not the same as healed.** The most common cause of complications is acting on surface appearance before the internal channel is mature.
- **Your piercer is your best resource.** A professional wants to see you at downsize and welcomes check-ins.

★ GLOSSARY

Terms Used in This Guide

Plain language definitions for every term you'll encounter.

Anodizing

An electrical process applied to titanium or niobium that creates a colored oxide layer on the surface. Anodized titanium is still implant-grade and biocompatible — the color is a surface characteristic, not a coating.

APP (Association of Professional Piercers)

An international nonprofit that sets and promotes professional standards for body piercing. Membership requires demonstrated knowledge of sterilization, materials, anatomy, and safe practice. Website: safepiercing.org.

ASTM F136

The specific certification grade for implant-grade titanium. Meets the American Society for Testing and Materials standard for surgical implant use. Always ask for this specifically.

ASTM F138

The specific certification grade for implant-grade steel. Contains approximately 10–14% nickel locked into the alloy. Not suitable for clients with nickel sensitivity.

Autoclave

A pressurized steam sterilization unit. The only method approved for sterilizing reusable piercing instruments. Cannot be replaced by UV light, chemical soaking, or spray disinfectant.

Bloodborne Pathogens (BBP) Certification

A safety certification covering proper handling of blood and bodily fluids. All professional piercers should hold a current certification.

Curved Barbell

A barbell with a slight arc. Appropriate for some placements (navel, rook, eyebrow) and inappropriate for others (surface piercings, where it causes rejection).

Dermal Anchor

A single-point surface piercing with a flat base plate under the skin and a removable decorative top. Not permanent — migrates or rejects over time.

Downsize

A follow-up appointment, typically 4–8 weeks after initial piercing, in which the starter jewelry is replaced with a shorter post or smaller diameter ring. A clinical step in healing, not optional.

Externally Threaded

Jewelry with screw threads on the outside of the post. Threads drag through the piercing channel during insertion, damaging healing tissue. Not appropriate for initial piercings per APP standards.

Fistula

The healed tissue channel that forms around a piercing over time. What you think of as the "piercing hole" is technically a fistula — a tube of epithelial tissue lining the channel.

Gauge

A measurement of jewelry thickness. A lower gauge number means a thicker piece. Earlobes: typically 20g–16g. Cartilage: 16g–14g. Nipples and navels: commonly 14g.

Hypertrophic Scar

A raised, firm scar that stays within the wound boundary. Most piercing bumps are hypertrophic scars — irritation responses that resolve when the irritant is removed.

Implant-Grade

A material designation indicating the metal meets standards for use in surgical implants and direct body contact. Requires a specific ASTM certification.

Internally Threaded

Jewelry with threads inside the post. The decorative end screws inward. Only smooth metal contacts the piercing channel. APP standard for initial piercings.

Keloid

A type of scar caused by genetic predisposition to overproduce collagen. Extends beyond the wound boundary and may continue growing after the wound heals. Distinct from hypertrophic scarring. Requires medical treatment.

Labret Stud

A flat-backed post with a single decorative front. The flat back sits flush against tissue. Used as a standard starter in many ear and facial piercings.

Lymph Fluid

The clear to slightly white-yellow fluid that drains from a fresh piercing and dries into crust. Completely normal. Not infection. Do not pick at it.

Migration

The movement of jewelry from its original placement position. May or may not lead to full rejection.

NeilMed Wound Wash

The most widely recommended sterile saline for piercing aftercare. 0.9% NaCl, no additives. Available at most pharmacies.

Niobium

An elemental, nickel-free metal with excellent biocompatibility. Can be anodized for color. An excellent option for clients with metal sensitivity.

Rejection

The process by which the body pushes jewelry toward and eventually through the skin surface. Recognized by thinning tissue over the jewelry bar, visible jewelry outline, and shift in position.

Spore Testing

Biological testing of an autoclave using heat-resistant spores to verify sterilization conditions are being achieved. APP-standard studios run tests weekly and keep records.

Starter Jewelry

The piece placed in a fresh piercing. Longer than final jewelry to accommodate initial swelling. Must be implant-grade, internally threaded or threadless, and appropriate in gauge.

Surface Bar (Staple Bar)

Jewelry specifically designed for surface piercings. Has a flat horizontal base with two short vertical posts at 90 degrees. Eliminates the inward pressure caused by curved barbells in surface placements.

Sweet Spot (Septum)

The thin membrane of soft tissue between the cartilage and the tip of the nose, through which a correctly placed septum piercing passes. Does not go through cartilage.

Threadless (Press-Fit)

Jewelry that uses a bent pin mechanism rather than threading. No threading contacts the tissue. APP-approved for initial piercings.

Vasovagal Response

A fainting or near-fainting response triggered by pain, anxiety, or the sight of blood. Eating before your appointment reduces risk. Tell your piercer if you have experienced this before.

★ PRINTABLE REFERENCE CARD

Questions to Ask Before You Commit

Screenshot this page. Take it to your appointment.

Q: What sterilization method do you use?

- ✓ Autoclave. Weekly spore tests on file.
- ✗ Disinfectant spray / UV / bleach

Q: Can I see the needle before you open it?

- ✓ Yes — opened from sealed sterile packaging in front of you.
- ✗ Already set up / pre-opened

Q: What material exactly is this jewelry?

- ✓ ASTM F136 titanium / ASTM F138 steel / solid 14k gold / niobium.
- ✗ It's hypoallergenic / surgical steel / not sure

Q: Is this internally threaded or threadless?

- ✓ Yes — all starter jewelry is internally threaded or press-fit threadless.
- ✗ Does it matter? / I don't know what that means

Q: Do you use a piercing gun for any piercings?

- ✓ Never. Only single-use sterile hollow needles for every piercing.
- ✗ Only for earlobes / guns are fine if cleaned

Q: Are you APP-member or do you follow APP standards?

- ✓ Yes — I'm an APP member / I follow APP standards.
- ✗ I've been piercing for years / What's the APP?

Q: What aftercare do you recommend?

- ✓ Sterile saline only. 0.9% NaCl. 1–2 times per day. Do not rotate.
- ✗ Alcohol / H2O2 / Bactine / tea tree / rotate twice a day

Q: Should I rotate my jewelry?

✓ Absolutely not. Ever. That advice is outdated and causes real harm.

✗ Yes, rotate twice a day / move it when cleaning

Q: Do you do downsize appointments?

✓ Yes, at 4–8 weeks.

✗ You don't really need to / do it yourself

Q: What should I do if I get a bump?

✓ Come back and see me. I'll assess the cause.

✗ Put tea tree oil on it / change the jewelry yourself

More than 2 red flag answers = find a different studio.

Find APP-verified piercers: **safepiercing.org**

PIERCING 101

What They Don't Tell You

"An informed client is the best client —
and the best thing that can happen to the professional piercing industry."

By Rick Angell

Based on APP Standards (safepiercing.org) and The Piercing Bible by Elayne Angel
Educational resource. Consult a licensed medical professional for health concerns.